



Protocol Intelligence Report

Structured protocol review, comparable cohort benchmarking, recommendations, and audit-ready reporting.

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Prepared for clinical development, trial design, and operational planning review.

Classification
Internal review draft
File: trial_protocol_report.pdf

1. Executive Summary

- Protocol title: Single Ascending Dose Study of Eicosapentaenoic Acid and Calcium L-S-Methyltetrahydrofolate
- Clinical focus: Healthy Subjects
- Study structure: Interventional | Not provided
- Profile status: Reviewed with medium extraction confidence
- Comparator evidence: 1000 matched studies | 715 completed | 69 disrupted.
- Decision posture: Mixed precedent posture (completed fit 54.2%, disrupted fit 48.4%).
- High priority: Finalize the missing core design fields before using this draft for governance review or external discussion.
- Monitor priority: Keep the enrollment target under active review as comparator evidence evolves.

2. Senior Decision Signals

Decision Area	Signal	Implication	Evidence
Evidence base	Strong	Confidence is highest when completed and disrupted comparators are both adequately represented.	1000 total studies 715 completed 69 disrupted.
Overall posture	Mixed precedent posture	This is the clearest top-line read on whether the draft is behaving more like successful or disrupted precedent.	Completed fit 54.2% Disrupted fit 48.4%.
Enrollment plan	Within completed-trial range	Enrollment targets outside the completed range need stronger feasibility justification.	Target 32 Completed IQR 21.0 to 72.0.

Decision Area	Signal	Implication	Evidence
Execution exposure	Lower observed disruption exposure	Higher disruption prevalence should trigger more explicit mitigation planning.	Disrupted-status share 6.9%.
Protocol completeness	Specification gaps remain	Missing design fields make the benchmark less trustworthy and reduce report defensibility.	Phase, Intervention Model
Lead action	High	Finalize the missing core design fields before using this draft for governance review or external discussion.	Missing or unspecified fields: Phase, Intervention Model.

3. Executive Action Register

Priority	Category	Action	Recommendation	Evidence
High	Protocol completeness	Clarify	Finalize the missing core design fields before using this draft for governance review or external discussion.	Missing or unspecified fields: Phase, Intervention Model.
Monitor	Enrollment posture	Monitor	Keep the enrollment target under active review as comparator evidence evolves.	Protocol target 32; completed-trial IQR 21.0 to 72.0; disrupted-trial IQR 0.0 to 44.0.

4. Reviewed Protocol Profile

Field	Value
Title	Single Ascending Dose Study of Eicosapentaenoic Acid and Calcium L-S-Methyltetrahydrofolate
Condition	Healthy Subjects
Sponsor	Sichuan Credit Pharmaceutical Co., Ltd
Study Type	Interventional
Phase	Not provided
Planned Enrollment	32
Arms Count	4
Allocation	Randomized
Masking	Open Label
Intervention Model	Not provided

Primary Purpose	Treatment
Comparator	Not provided
Target Population	Healthy Subjects
Geography Focus	China
Start Date	2026-01-21
Completion Date	Not provided
Endpoint Focus	Not provided
Primary Endpoints	Pharmacokinetic dose linearity
Secondary Endpoints	Safety and tolerability

5. Comparative Narrative

Matched 1000 studies for Healthy Subjects, including 715 completed comparators and 69 disrupted comparators. Top-line posture: Mixed precedent posture (completed fit 54.2%; disrupted fit 48.4%). Enrollment position: Within completed-trial range. Protocol target 32 | completed-trial IQR 21.0 to 72.0 | disrupted-trial IQR 0.0 to 44.0. Most important design differential: Primary Purpose shows 37.2% completed-trial match versus 39.1% disrupted-trial match. Lead action: Finalize the missing core design fields before using this draft for governance review or external discussion. Interpretation note: this output is intended to support senior design review, not replace clinical, statistical, operational, or regulatory judgment.

6. Comparator Cohort Definition

Metric	Value
Clinical condition	Healthy Subjects
Matched trial count	1000
Completed comparators	715
Disrupted comparators	69
Active comparators	121
Evidence strength	Strong
Risk-status share	6.9%
Median site count	1.0
Median country count	1.0

7. Success vs Disruption Benchmark

Domain	Protocol Draft	Completed Precedent	Disrupted Precedent	Signal
Enrollment	32	Median 38.0 IQR 21.0 to 72.0	Median 14.0 IQR 0.0 to 44.0	Within completed-trial range
Planned Duration	Not provided	Median 7.0 mo IQR 2.8 to 17.5 mo	Median 18.4 mo IQR 8.8 to 31.4 mo	Insufficient evidence
Phase	Not provided	Not available	Not available	Protocol field not specified
Study Type	Interventional	88.4%	76.8%	Mixed precedent
Allocation	Randomized	73.6%	63.8%	Mixed precedent
Masking	Open Label	0.0%	0.0%	Weak precedent signal
Intervention Model	Not provided	Not available	Not available	Protocol field not specified
Primary Purpose	Treatment	37.2%	39.1%	Mixed precedent
Endpoint Focus	Other	72.0%	62.3%	Mixed precedent

8. Decision Scorecard

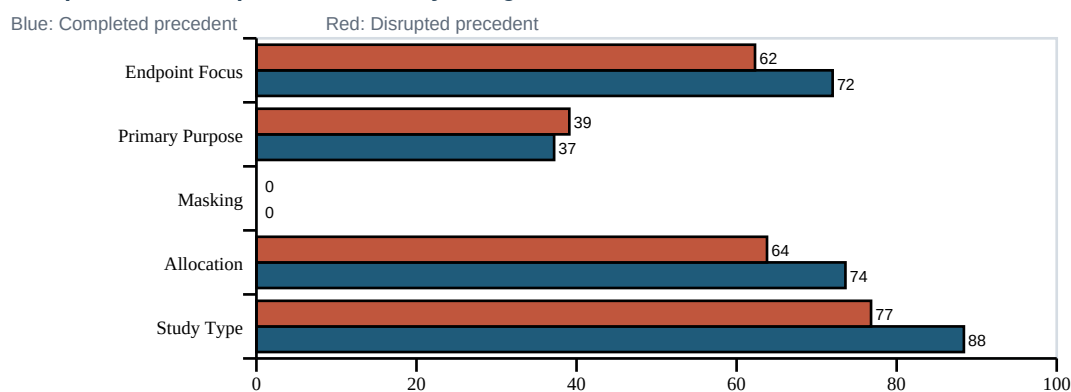
Metric	Value
Matched cohort size	1000
Completed comparator count	715
Disrupted comparator count	69
Evidence strength	Strong
Completed precedent fit (%)	54.2
Disrupted precedent fit (%)	48.4
Net precedent gap (%)	5.8
Overall posture	Mixed precedent posture
Enrollment target	32
Completed enrollment IQR	21.0 to 72.0
Disrupted enrollment IQR	0.0 to 44.0
Completed median duration (months)	7.0
Disrupted median duration (months)	18.4
Median site count	1.0

9. Design Differential Matrix

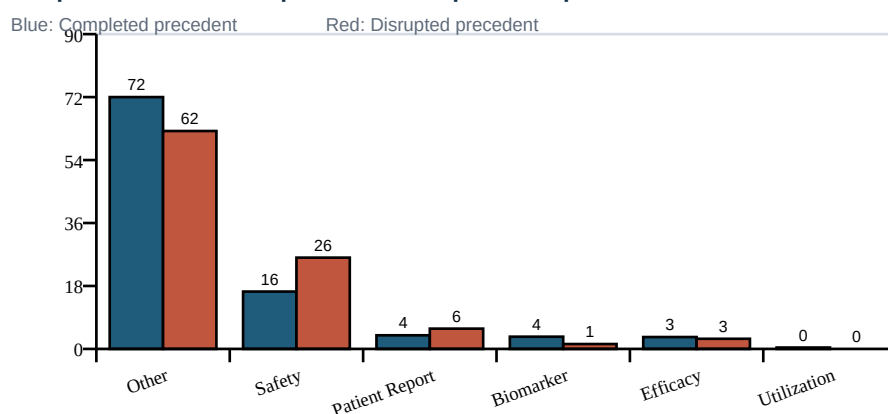
Domain	Protocol Choice	Completed %	Disrupted %	Gap %	Signal
Phase	Not provided	Not available	Not available	Not available	Protocol field not specified
Study Type	Interventional	88.4	76.8	11.6	Mixed precedent
Allocation	Randomized	73.6	63.8	9.8	Mixed precedent
Masking	Open Label	0.0	0.0	0.0	Weak precedent signal
Intervention Model	Not provided	Not available	Not available	Not available	Protocol field not specified
Primary Purpose	Treatment	37.2	39.1	-1.9	Mixed precedent
Endpoint Focus	Other	72.0	62.3	9.7	Mixed precedent

10. Key Figures

Completed vs Disrupted Precedent by Design Domain



Endpoint Focus in Completed vs Disrupted Comparators



11. Recommendation Detail

Priority	Category	Recommendation	Rationale	Evidence
High	Protocol completeness	Finalize the missing core design fields before using this draft for governance review or external discussion.	Key design fields are still unspecified, which weakens benchmark interpretation and report defensibility.	Missing or unspecified fields: Phase, Intervention Model.
Monitor	Enrollment posture	Keep the enrollment target under active review as comparator evidence evolves.	The target sits inside both completed and disrupted benchmark ranges, so execution quality and site strategy will matter more than the number alone.	Protocol target 32; completed-trial IQR 21.0 to 72.0; disrupted-trial IQR 0.0 to 44.0.

12. Comparator Exemplars

Comparator Lens	Status	NCT ID	Title	Enrollment	Sponsor
Completed Precedent	Completed	NCT06459258	Throwing Velocity and Patient Reported Outcomes in Elite Level Handball Players After Completion of Shoulder-Pacemaker Strength Training	14.0	Sportorthopädie Zentrum in Wien Hietzing
Completed Precedent	Completed	NCT03506126	Requirement for Leucine in Adults Over 60 Years	16.0	The Hospital for Sick Children
Completed Precedent	Completed	NCT04371185	Comparative Study to Evaluate the Pharmacokinetics of BAT2206 vs Stelara® in Healthy Subjects	270.0	Bio-Thera Solutions
Completed Precedent	Completed	NCT03150108	Study of rhPTH(1-84) in Japanese Healthy Subjects Compared With Matched Caucasian Healthy Adult Subjects	24.0	Shire
Disrupted Precedent	Suspended	NCT06063759	Comparative Measurement of Microcirculation and Other Parameters With the Vital USA VitalDetect™ in Patients With Diabetes Mellitus Type 1 and Type 2 Versus Healthy Participants	100.0	Sciema UG
Disrupted Precedent	Terminated	NCT02363946	A Study of ARC-AAT in Healthy Volunteer Subjects and Patients With Alpha-1 Antitrypsin Deficiency (AATD)	65.0	Arrowhead Pharmaceuticals
Disrupted Precedent	Withdrawn	NCT06385938	Ultrasound Imaging and Quantitative Vibro-Acoustic Assessment of Rickets Under the Age of Ten	0.0	Mayo Clinic
Disrupted Precedent	Withdrawn	NCT00376285	Effects of Fenofibrate on the Safety, Tolerantion and Efficacy Biomarkers of CP-742,033	0.0	Pfizer
Active Context	Recruiting	NCT05847855	Plasma cfDNA Fragmentomics for Early pNET Detection and Differential Diagnosis of Solid Pancreatic Tumors	1000.0	Fudan University
Active Context	Recruiting	NCT06814717	A Safety, Tolerability and Pharmacokinetic Study of cP12 in Adults With Up To 10% Total Body Surface Area Burns	6.0	Neomatrix Therapeutics, Inc.

14. Audit Trail and Provenance

Timestamp	Actor	Action	Details
2026-03-14T20:57:54.160572m	System	extract_protocol_profile	Extracted protocol profile from Prot_SAP_000 (1).pdf.
2026-03-14T20:58:02.736294Z	Reviewer	review_protocol_profile	Reviewer updated and confirmed protocol profile fields.
2026-03-14T20:58:13.407447m	System	build_comparable_cohort	Built registry comparison cohort for Healthy Subjects.
2026-03-14T20:59:12.407382m	System	export_pdf_report	Generated trial_protocol_report.pdf.

15. Methodology Notes

Source trials were retrieved from ClinicalTrials.gov and normalized into a comparator cohort. This report separates completed comparators from disrupted comparators and uses those lenses to assess how the reviewed draft aligns with stronger or weaker precedent patterns. Protocol fields were derived from uploaded documents using deterministic heuristics and, when configured, an LLM-assisted structured extraction step. All outputs are intended to support expert review rather than replace clinical, statistical, operational, or regulatory judgment.